جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : شادية عبد المتجلي ابوشامة محمد

المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** 34 ش الغزازي العمرانيه

رقم البطاقة : 26204232100829 **رقم التليفون :** 01116833220 **تاريخ تسجيل الإستمارة :** 10-10-2026

العمر(تاريخ الميلاد) : 23-04-1962 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:(----------)**

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: شادية عبد المتجلي ابوشامة محمد
National ID: 26204232100829

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

09/15/2026

MDT FORM

Name:	Shadia Abd Elmotgaly Abo Shama .	**MRN:**	651
Age:	64 Y	**ID:**	26204232100829
Menopausal Sta	POST	**Phone:**	01116833220
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:		**Reason Of MDT:**	POST OP

Clinical Data:

BX: IDC GII

ER: 8/8 **PR:** 6/8 **HER2:** +3 **KI67:**

Sonomammography:
RT WLE + SLNB
T0 N0
FREE MARGINS
FREE LNS 0/10

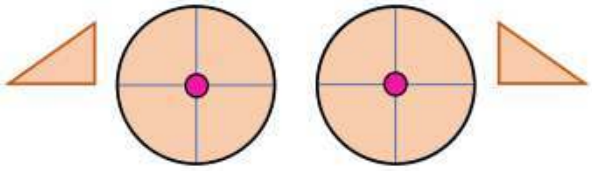
Mets Workup:

History:
63 Y POSTM RT BREAST CANCER CT2 N1 M0 CLIPS INSERTED IDC2 ER8/8 PR6/8 HER23+ PT REC 11WKS TAXOL CARBOPLATIN, THEN CARBOPLATIN OMITTED, PT CONTINUED TAXOL SINGLE AGENT TILL 18 WEEKS, LAST ON 14/4, AND 5 CYCLES PHESGO, LAST 14/4

Recommendations:

MDT Outcome & Plan
For PORT
Continue Phesgo up to 1 year
Arimidex x 5 years + Zometa / 6m

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د خالد عبد العزيز	د محمد نيراوي				

GUSTAVE ROUSSY INTERNATIONAL AGENCY
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسى الترناشونال ـ مصر معمل الباثولوجى
جوستاف روسي

Patient Name	شاديه عبدالمتجلى ابو شامه	**Case No.**	**357757**
Sex/Age	**Female 64 Y**	**Patient No.**	**25841**
Registered Date	**12/8/2026 03:46:09 PM**	**Referred By Dr.**	**Surgery**
Validation Date	**07/9/2026 01:45:05 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**12/9/2026 12:46:08 PM**	**Pathology No.**	**2565/26**

Pathology Report

Clinical Data: Right breast cancer, Post NACT.

Nature of the specimen: Breast conservative cancer and sentinel lymph node biopsy.

Gross Examination:
Wide local excision measuring 10x5x2 cm covered with skin ellipse measured 3.5x1.5 cm.
On serial sectioning revealed wire guided tumor bed site measuring 2.5x2x1 cm.
Distance from margins: 0.3 cm from medial, 0.5 cm from lateral, 0.2 cm from superior, 0.1 cm from inferior, 6 cm from deep and 1 cm from skin.
* Fibrofatty tissue piece measured 8x5x1.5 cm (wired) showing clipped nodule 3x2 cm.
Sentinel lymph nodes: Fibrofatty tissue piece measuring 10x5x1 cm, containing 10 lymph nodes, the largest is 2x2x0.5 cm.
*** Frozen section:**
- Free margins.
- Clipped breast tissue: free of tumor.
- Free sentinel lymph nodes (0/5), 10 nodes detected but only 5 subjected to frozen section.

*** Block designation:** 1,2- TBS, 3-medial margin, 4-lateral margin, 5-superior margin, 6-inferior margin,7-deep margin,8-skin, 9- Clipped tissue, 10, 11 lymph nodes.
*** Grossing by: Dr. Aya Ezzat.**

Microscopic examination:

- **Histological type:** No residual viable tumor cells.
- **Ductal carcinoma in situ:** Absent.
- **Lympho-vascular emboli:** Absent.
- **Skin:** Free.
- **Clipped area:** Free of tumor cells or nodal tissue.
- **Lymph node involvement:** Free sentinel lymph nodes of metastatic deposits (0/10).
- **Resection Margins:** Free surgical margins.
- **Histologic response to therapy:** Pathological complete response.

Diagnosis:
Right breast cancer (Post NACT), Wide local excision and Sentinel Lymph node biopsy:
No residual viable tumor cells in a previously diagnosed invasive duct carcinoma.
Resection margins: Free of tumor cells

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL

Gustave Roussy International Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي

جوستاف روسي

Patient Name	شاديه عبدالمتجلى ابو شامه	**Case No.**	357757
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Registered Date	12/8/2026 03:46:09 PM	**Referred By Dr.**	Surgery
Validation Date	07/9/2026 01:45:05 PM	**Branch**	Gustave Roussy
Reporting Date	12/9/2026 12:46:08 PM	**Pathology No.**	2565/26

Skin: Free
Sentinel lymph nodes: Negative for metastatic deposits (0/10).
Stage: ypT0, ypN0

Signature
Dr. Walaa Ibrahim Ghoneim, MD.

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

ame	الأستاذه / شاديه عبدالمتجلي ابو شامه محمد :	Request No.	: 13003580
atient No.	: 26204232100829	Request Date	: 20/08/2025
Path No.	: SR-1149/25	Verification Date	: 21/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 63 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:
One referred block labelled by (202899-25). Together with, three immunostained slides for (ER, PR & HER2neu).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
* Focal intermediate grade DCIS (Solid & cribriform patterns).
* Angioinvasion: Absent.
* Perineural invasion: Absent.

* IPT:
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).

Diagnosis:
Right breast mass, Referred block & slides (Tru-cut biopsy):
- Invasive duct carcinoma, Grade 2, with focal intermediate grade DCIS (Solid & cribriform patterns).
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 6/8).
- Her2: Positive (score 3+).

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / شاديه عبدالمتجلي ابو شامه محمد	Request No.	: 13003580
Patient No.	: 26204232100829	Request Date	: 20/08/2025
Path No.	: SR-1149/25	Verification Date	: 21/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 63 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

PATIENT NAME: SHADIA Abdalmotgaly ABOSHAMH MOHAMED
GENDER: Female
DATE : 10-Aug-2025 09:58 AM

PATIENT ID: 20225021754
AGE: 63 y
ORDER: Z232426,Z232429

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC ASSESSMENT TO THE LIVER

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 70 ml.

FINDINGS: A reported case of right breast cancer (IDC)

1-CHEST

- Right breast spiculated mass lesion with surrounding smudging fats, it is seen roughly measuring about 25 x 36 mm , it is seen far away the skin by 4 mm, with multiple ipsilateral enlarged axillary LNs the largest one is seen measuring about 17 x 13 mm.
- Few left prominent axillary LNs are noted, the largest one is seen measuring about 9 x 6 mm.
- Few prominent right internal mammary LNs are noted.
- Few bilateral scattered millimeteric pulmonary nodules, the largest one is seen pleural based at left lung lower lobe posterior segment reaching up to 6 mm .
- Tiny pleural tag is seen at right lung lower lobe posterior segemnet reaching up to 4 mm.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

مركز بهية الهرم
٩ شارع علوية أول شارع الهرم بجوار داري، الجيزة
مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة
www.Baheya.org
16602

CT Scan
[[DOCTYPE:CT_SCAN]]

PATIENT NAME: SHADIA Abdalmotgaly ABOSHAMH MOHAMED
GENDER: Female
DATE : 29-Jul-2025 09:41 AM

PATIENT ID: 20225021754
AGE: 63 y
ORDER: Z226566,Z226567

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- **Right breast lump.**
- **Positive family history (Sister).**

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound.

REPORT:

- Scattered fibroglandular densities (ACR B).
- Bilateral loosely attached punctate calcification.

Right breast

- **LIQ spiculated highly dense mass lesion away from the nipple about 4.5cm in CC view.**
- **UOQ rounded equal dense nodule.**

Complementary Ultrasound shows:

- **LIQ at 3-4 o'clock show spiculated vascular hypoechoic mass lesion with surrounding desmoplastic reaction, measures 1.8x2.4cm abutting the overlying skin. Associated with skin thickening reaching to 3mm.**
- **UOQ at11 o'clock show pathologically enlarged IMLN with effaced hilum.**
- Normal nipple areola complexes.
- **The right axilla show multiple (3 in number) pathological lymph nodes, the largest measures 18.5x10.5mm.**
- No pathologically enlarged infra or supra-clavicular LNs

Left Breast

- UOQ rounded equal dense nodule. Complementary US shows IMLN.
- No spiculated mass lesions or grouped microcalcifications.

Complementary Ultrasound shows:

- Few scattered small cysts.
- Dilated retro-arolar ducts with avascular echogenic contents.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- **The left axilla & IMLN show multiple indeterminate lymph nodes, the largest measures 14.5x6.5mm.**

OPINION:

- **Right LIQ malignant looking lesion with pathologically right axillary lymph nodes (BIRADS 5) for histopathological correlation.**
- **Left multiple axillary indeterminate LNs for FNAC.**
- Left benign findings. However, no suspicious findings are noted (BIRADS-2).

مركز بهية الهرم
٩ شارع علوية أول شارع الهرم بجوار نادي الجزيرة
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٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة
www.Baheya.org
16602
Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient ID: 91715	Patient Name English: Shadia abdalmtgli aboshama
Age: ٦٢ سنة, ٧ شهر	2025/12/22 01:25:58 PM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: Pathologically proven right breast IDC, grade II, triple positive molecular subtype, on NACT.

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: minimal background enhancement

ACR: b (scattered fibro-glandular densities)

LEFT BREAST:

- **Low energy (Non contrast) images:** show no masses, asymmetry or distortion or suspicious microcalcifications. Evidence of benign macrocalcifications.
- **Recombined (Post contrast) images:** show no pathological mass or non-mass enhancement.
- Normal skin / nipple / areolar complex.
- **Complementary US:** 12 o'clock, para-areolar duct ectasia with clear content reaching 7 mm in caliber.

RIGHT BREAST:

- **Low energy (Non contrast) images:** inner central, middle third, irregular speculated equal density mass lesion with tissue marker within, surrounding architectural distortion and overlying focally thickened skin are noted.
- UOQ, middle third, 4 cm from nipple, another tissue marker is seen with no definite related densities.
- **Recombined (Post contrast) images** show inner central, 2.5 x 1.7 x 1.5 cm heterogeneously enhancing mass lesion seen 3.2 cm from nipple.
- No otherwise pathological uptake.
- Normal skin / nipple / areolar complex.
- **Complementary ultrasound:** 3 o'clock correlate malignant mass lesion seen touching the skin that showed focal thickening reaching 5 mm with tissue marker within.
- UOQ, 11 o'clock, intra-mammary lymph node showed non-specific sonographic appearance and tissue marker within.
- Indeterminate level 1 three axillary lymph nodes. No pathological infra or supra-clavicular lymph nodes.

IMPRESSION:

LEFT BREAST: (BIRADS 2= Benign findings) benign macrocalcifications and duct ectasia.

RIGHT BREAST: (BIRADS 6= pathologically proven malignant) Partial radiological response of the previously reported malignant process.

Best regards,

Dr. Amal Hegazi, M.Sc.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient's Name: Shadia I gali Abu Shama

Date: 2026-06-06

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt

FOLLOW UP BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION BREAST SONOGRAPHY REVEALED:

CLINICAL SUMMARY:

A known case of right carcinoma, under NACT and target therapy, for follow up

FINDINGS:

RIGHT BREAST:

- Dense parenchyma, (ACR c).
- The UOQ and LIQ are still the seat of the two previously inserted clips, with no definite underlying masses.
- Complimentary U/S demonstrated no definite masses.
- No definite pathological axillary nodes are seen in today's study.
- Clear infra and supraclavicular regions.

LEFT BREAST:

- Dense parenchyma, (ACR c).
- No mass, spiculation or suspicious micro calcifications, this was confirmed by an U/S examination.
- Normal skin/nipple/areola complex.
- Non specific axillary nodes are noted.

CONCLUSION:

Right breast (BIRADS 6): no residual mass is seen denoting complete response.

Left breast (BIRADS 1): normal study

Best regards,
Dr. Yara Mohamed, MD

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY INTERNATIONAL CANCER CAMPUS

جوستاف روسي انترناشونال كامبوس

ECHO REPORT

NAME: سادية عبدالمنجلي ابوشامة DATE: 14/4/2026

ID NUMBER: / SEX: ♀ AGE: 64

EF	65%	55-75%	PWT	1.0	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.5	3-3.7cm
LVESD		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions ⌐ ame

LV Wall Thickness ⌐ amd

LV Diastolic Function DD grade II.

SWMA ⌐ o SWMA

LA and Ao Root Dimensions ⌐ amd

Mitral Valve Deg. M.V with mild MR

Aortic Valve Scl. A.V with no sig systolic gradient

Tricuspid Valve mild TR

Pulmonary Valve ⌐ amd

Right Side and Systolic Function ⌐ amd

Intracardiac Masses and Effusion ⌐ o

DR.

Echo
[[DOCTYPE:ECHO]]

Patient's Name:	Shadia A gali Abu Shama
Date:	2026-06-06
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

FOLLOW UP BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION BREAST SONOGRAPHY REVEALED:

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A known case of right carcinoma, under NACT and target therapy, for follow up

FINDINGS:

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- No definite pathological axillary nodes are seen in today's study.
- Clear infra and supraclavicular regions.

LEFT BREAST:
- Dense parenchyma, (ACR c).
- No mass, spiculation or suspicious micro calcifications, this was confirmed by an U/S examination.
- Normal skin/nipple/areola complex.
- Non specific axillary nodes are noted.

CONCLUSION:
Right breast (BIRADS 6): no residual mass is seen denoting complete response.
Left breast (BIRADS 1): normal study

Best regards,
Dr. Yara Mohamed, MD

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name : Shadia Abdel-Mutajalli Abu Shama
Branch : Dokki
Date : 05-09-2025 12:37
Medical No. : 195061

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- 63-year-old lady, with history of right breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen allover the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Shadia Abdalmtgli Abu Shama Mohamed	Date : 12/11/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 63 Year(s)
Medical ID: 2346/2025	Pat. ID: 132888

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of right breast mass

1. ***Chest:***

- Right breast speculated calcified mass measures 2 x1.9 cm .
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 1.6x 1 cm.
- Left cardiophrenic lymph node reaching 0.6cm.
- Few bilateral scattered tiny pulmonary nodules ranging from 3-4 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Small calcified subserous uterine myoma .
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. ***The scanned skeleton:***

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

)PINION:

- ***Right breast mass .***
- ***No pulmonary , hepatic or bony metastatic deposits .***

Dr/Reham Khedr ,,,MD

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL

جوستاف روسي انترناشونال

ECHO REPORT

NAME: شاوية كمالية

DATE: 9/8/2026

ID NUMBER: /

SEX: AGE:

EF	62%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.7	2-4cm	LA	3.6	2-4cm
FS	28%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GE

SWMA No RWMA

LA and Ao Root Dimensions Normal

Mitral Valve

Aortic Valve N

Tricuspid Valve

Pulmonary Valve N

Right Side and Systolic Function TAPSE: 2.1

Intracardiac Masses and Effusion

DR.

Echo
[[DOCTYPE:ECHO]]